

COMPREHENSIVE DISCHARGE SUMMARY

UNIVERSITY MEDICAL CENTER Department of Hematology/Oncology

DISCHARGE SUMMARY

Patient: David

Chen **MRN:** AML003 **DOB:** 01/30/1949 **Admission:** 03/08/2024 **Discharge:** 03/15/2024 **Attending:** Rebecca Liu, MD

CHIEF COMPLAINT: Febrile neutropenia

HISTORY OF PRESENT ILLNESS: Mr. Chen is a 75-year-old gentleman with AML (ELN adverse risk, IDH2 R140Q and DNMT3A mutated) diagnosed February 2023, currently on cycle 13 of venetoclax/azacitidine, who presented with fever to 102.3°F, rigors, and mild productive cough x2 days.

PAST MEDICAL HISTORY:

1. AML as above
2. Type 2 DM (HgbA1c 7.2%)
3. Hypertension
4. BPH
5. Hyperlipidemia
6. Remote CVA 2019 (no residual deficits)

ONCOLOGIC HISTORY: Presented 02/10/23 with fatigue and easy bruising. Initial labs: WBC 45.6K (78% blasts), Hgb 7.1, Plt 23K. Bone marrow biopsy confirmed AML with 82% blasts. NGS revealed IDH2 R140Q (VAF 38%), DNMT3A R882H (VAF 42%). Cytogenetics: normal karyotype. Started venetoclax 400mg daily (after 3-day ramp-up) + azacitidine 75mg/m2 days 1-7 on 02/18/23.

Response Assessment:

- Cycle 2 Day 28: Hypocellular marrow, 3% blasts (morphologic CR)
- Cycle 4: MRD negative by flow cytometry
- Cycle 8: Continued CR, mild cytopenias
- Most recent (Cycle 12, 02/2024): Maintained CR, MRD negative

TREATMENT COURSE: Has tolerated therapy remarkably well with dose modifications:

- Venetoclax reduced to 200mg (cycles 5-8) for prolonged neutropenia
- Currently back on 400mg with growth factor support
- Azacitidine maintained at full dose

HOSPITAL COURSE:

Day 1-2: Admitted through ED with ANC 420, fever 102.3°F. Blood cultures x2 drawn. Chest X-ray showed possible RLL infiltrate. Started on cefepime 2g IV q8h and vancomycin 1g IV q12h. Urine culture negative. Added azithromycin 500mg daily for atypical coverage.

Day 3-4: Blood cultures grew *Streptococcus viridans* (1/2 bottles), sensitive to penicillin. De-escalated to ceftriaxone 2g daily. Fever curve improving. ANC recovering (680 on day 4).

Day 5-7: Afebrile x72 hours. ANC 1,240. Repeat blood cultures negative. Transitioned to amoxicillin-clavulanate 875mg PO BID to complete 10-day course. Venetoclax held during admission, resumed on day 6.

LABORATORY DATA AT DISCHARGE:

- WBC 2.8K (ANC 1,450)
- Hgb 9.2 g/dL
- Plt 68K
- Creatinine 1.1 mg/dL
- ALT 28, AST 31
- Bilirubin 0.8 mg/dL

DISCHARGE MEDICATIONS:

1. Venetoclax 400mg PO daily (resume 03/16)
2. Azacitidine - next cycle due 03/22/24
3. Amoxicillin-clavulanate 875mg PO BID x4 more days
4. Acyclovir 400mg PO BID
5. Fluconazole 200mg PO daily
6. Metformin 1000mg PO BID
7. Lisinopril 10mg PO daily
8. Atorvastatin 40mg PO daily
9. Tamsulosin 0.4mg PO daily
10. Pantoprazole 40mg PO daily

DISCHARGE INSTRUCTIONS:

- Complete antibiotic course
- Resume venetoclax tomorrow (03/16)
- Return for azacitidine cycle 14 on 03/22
- CBC on 03/19
- Call if fever >100.4°F, worsening cough, or other concerns

FOLLOW-UP:

- Hematology clinic 03/22/24 for cycle 14 azacitidine
- Primary care in 2 weeks

PROGNOSIS: Mr. Chen continues to have an excellent response to venetoclax/azacitidine after 13 months of therapy with maintained MRD-negative remission. Given his age and adverse risk features, plan is to continue current therapy indefinitely as long as tolerated. We discussed potential for enasidenib given his IDH2 mutation should he progress, though he remains in deep remission currently.

DISCHARGE CONDITION: Stable, ambulatory

Rebecca Liu, MD Attending Hematologist

